

Tobacco Cessation: if then rules

Tobacco cessation — if-then rules

Decision rules the chatbot can pattern-match against to give specific, non-generic guidance to people who smoke or are quitting. Derived from the WHO *Be He@lthy Be Mobile — mTobaccoCessation handbook*, the QuitNowTXT message library, the *Gambia cessation clinical guidelines* (2016), and WHO *FCTC Article 14* implementation guidelines.

Pharmacotherapy specifics (drug names, doses, contraindications) are intentionally kept high-level. Specific prescribing decisions belong with a clinician, not the chatbot.

Stage of change — what to recommend

If the person says ...	Then ...
"I don't want to quit."	Don't push. Acknowledge respectfully. Offer one piece of information (one health fact, one money fact, or both) and leave the door open. "If anything changes, I'm here."
"I might quit, but not now."	Validate. Offer to explore one motivator: financial savings, children, breathing, fitness, a specific health concern they raised. Ask if they'd like to learn about reducing harm in the meantime.
"I want to quit within the next month."	Walk through Quit Day planning: pick a date, identify triggers, plan substitutes, tell a supporter, remove tobacco from home/car/bag, list reasons.
"I want to quit right now."	Quit Day is today. Run the Quit Day morning script (see Chat 2 example): water, breathing, distraction, get rid of tobacco. Set up daily check-ins.
"I quit last week" / "I quit yesterday"	Acknowledge the milestone. Ask how they're feeling. Run withdrawal triage (see below).
"I slipped, I had one cigarette."	Reframe slip vs. relapse. Identify the trigger. Encourage immediate re-commitment without shame.
"I started smoking again, fully."	Don't downplay it, don't catastrophize it. Validate the difficulty. Don't lecture. Ask what happened. When appropriate, offer to plan another Quit Day.

Nicotine dependence — interpreting Q-1 of the Fagerström screen

The single best dependence indicator is **how soon after waking** the person has their first cigarette.

Time to first cigarette after waking	Dependence level	Implications
Within 5 minutes	Very high	Strongly recommend pharmacotherapy. Withdrawal will be intense. NRT alone may be insufficient — flag for clinician discussion of bupropion or varenicline.
6-30 minutes	High	Pharmacotherapy strongly recommended. NRT (combination patch + gum) is usually adequate.
31-60 minutes	Moderate	Pharmacotherapy recommended, particularly for those with prior failed attempts.
After 60 minutes	Low	Many can succeed with behavioural support alone; offer NRT as an option, not a default.

Withdrawal symptom triage (first 1-2 weeks post-quit)

If the person reports ...	Then ...
intense cravings, multiple per day	Normal at this stage. Walk through the 5-D's (Delay, Deep breath, Drink water, Distract, Discuss). Recommend NRT if not already using.
irritability, mood swings	Normal, peaks around day 2-4. Recommend telling household members "this is the medicine working, not me", short walks, sleep priority.
can't sleep, restless	Common in first 1-2 weeks. Avoid caffeine after midday. Try a wind-down routine and limit screens before bed. Usually self-resolves.
headaches	Common. Drink water, paracetamol if needed, usually resolves within days.
constipation	Common, often missed. Recommend more water, fibre, walking. Self-resolves over 2-3 weeks.
weight gain in first weeks	Validate it's normal (most people gain 2-5 kg). Healthy snacks, water before meals, more walking. The cardiovascular gain outweighs the weight. Discourage stopping the quit attempt to "lose weight first".
extreme low mood, hopelessness, suicidal thoughts	Escalate. Recommend they speak to a health worker or mental-health professional today . Some people with prior depression have severe mood drops after quitting and may need temporary support or medication adjustment. Do not dismiss.
chest pain or severe shortness of breath	Not typical of withdrawal. Recommend they seek medical evaluation — this may be an unrelated cardiac issue surfacing.

Pharmacotherapy — when to suggest

If the person ...	Then ...
smokes $\leq$ 10/day, no prior quit attempts	Behavioural support first; NRT optional.
smokes > 10/day	NRT or other pharmacotherapy is strongly recommended.
has tried to quit before without medication and relapsed	Strongly recommend medication this time.
has had a heart attack within the past 2 weeks	NRT requires clinician judgment. Recommend they discuss with their cardiologist.
is pregnant or breastfeeding	Recommend non-pharmacological support first; any medication only on clinical advice. The risk of continuing to smoke during pregnancy is generally higher than the risk of NRT, but the decision must come from a clinician.
has uncontrolled high blood pressure	Recommend they get the BP controlled and discuss NRT with their clinician. Smoking is still the bigger risk.
asks about e-cigarettes/vaping	Be honest: less harmful short-term than cigarettes but not harmless, not a first-line recommendation, keeps nicotine addiction alive. Use only on specific clinician advice after other options.

Trigger management

Trigger pattern	Substitute / strategy
First cigarette of the morning	Change the morning ritual: shower first, drink water before tea, eat breakfast before going outside, do 5 push-ups, anything to break the script.
Cigarette with morning tea/coffee	Switch beverage temporarily (cold water, juice). After 2-4 weeks, reintroduce the original beverage in a different setting.
Cigarette after meals	Stand up and leave the table immediately. Brush teeth. Go for a 5-minute walk.
Cigarette during work breaks	Take the break, but in a different physical location (away from the smoking area). Bring water or a snack. Call a friend instead.
Cigarette when stressed	Practise the 4-7-8 breath (in 4, hold 7, out 8). Walk for 10 minutes. Text a supporter. Stress will not be made worse by not smoking — it will improve.
Cigarette when drinking alcohol	For the first 1-3 months: reduce or avoid alcohol, or only drink in places without smokers. Cravings are the strongest while drinking.
Cigarette around a smoking friend / family member	Tell them you've quit and ask them not to offer or smoke around you for now. If they won't respect that, limit time together for a few months.
Cigarette in the car	Remove the ashtray and lighter. Keep gum or sunflower seeds in the cup-holder. Sing along to music. Plan longer drives in smaller chunks with breaks.

Special populations

If the person is ...	Then ...
pregnant	Strongly recommend quitting immediately. Smoking in pregnancy causes low birth weight, premature birth, stillbirth, SIDS. Non-pharmacological support is first-line; any medication only on clinician advice.
breastfeeding	Strongly recommend quitting. Nicotine passes into breastmilk. If absolutely unable to quit, recommend smoking immediately after a feed (not before) to minimise infant exposure.
a teenager	Use peer-relevant framing (skin, sports performance, breath/smell, money, vaping risks). Do not lecture. Offer to connect them with a youth-friendly service.
living with someone who smokes indoors	Address the second-hand smoke issue. Recommend they have the “outside only, please” conversation. Their quit attempt will be much harder otherwise.
a hookah/shisha user only	Treat as a tobacco user. Many hookah users don’t think of themselves as smokers and underestimate the risk. Be specific about session-equivalent exposure (≈100 cigarettes per typical session).
a smokeless tobacco user (chewing, snus, etc.)	Same advice as cigarette cessation. Specific cancer risks: mouth, throat, pancreas. Encourage health-worker check of mouth and throat.
a recovering addict from another substance	Acknowledge they already have skills (relapse prevention, support networks). Connect quitting tobacco to those existing skills rather than framing it as a separate fight. Discuss any cross-trigger concerns with their addiction counsellor.

Relapse-prevention rules

If it is ...	Then ...
Quit Day	Daily check-in. Validate the milestone. Run the morning script.
Day 2-7	Daily check-in. Withdrawal triage as needed. Reinforce that this is the hardest week and it gets easier from here.
Week 2-4	Check in every few days. Cravings are still common; focus on trigger management. Begin weighting the long-term framing (“you’re past the worst part”).
Month 2-3	Weekly check-in. Most relapses happen in this window. Probe specifically for social and alcohol triggers, life events.
Month 4-6	Monthly check-in. Acknowledge milestones at 3 months and 6 months.
Month 6+	Less frequent. Frame as “ex-smoker.” Offer to re-engage if a stressful life event arises (loss, job change, illness).

Things to never do in the conversation

- Never moralise or shame someone for smoking, slipping, or relapsing.
- Never claim a specific cigarette type, brand, or alternative (“light”, “natural”, e-cigarette, shisha) is safe. None are.
- Never recommend a specific dose of pharmacotherapy — that is a clinician’s call.
- Never minimise withdrawal symptoms. Acknowledge them as the price of healing.
- Never dismiss reports of severe low mood or suicidal thoughts after quitting — escalate to clinical support.
- Never give up on someone who has relapsed multiple times. Most successful ex-smokers needed several attempts.